

Q How will my premature baby be fed on the neonatal intensive care unit?

This depends on how premature your baby is. The coordinated movement of sucking, breathing, and swallowing doesn't kick in until around 34 weeks' gestation, so your baby may need to be fed through an IV or tube at first.

Babies born before 34 weeks' gestation need to be fed either directly into their blood stream (an IV), or via a tube into their stomachs.

There are two ways that premature babies can be fed if they're too small to be breast- or bottle-fed. One way is through a vein, with an IV. Babies who are a bit older may have breast milk (or a suitable alternative) fed to them through a tube that goes to the stomach through the nose or mouth. Babies can be able to feed by breast or bottle, but need extra calories and nutrition of supplemental feedings.

If your baby needs to stay on a ventilator or doesn't yet have a coordinated sucking, breathing, and swallowing movement, she will receive milk through the tube. The milk feedings will be small, but frequent—once every two to three hours—and you may be able to hold your baby while she is being fed, and perhaps help to administer the milk. You can express your breast milk for your baby, or she can receive donor breast milk from a bottle bank, or enriched low-birthweight formula.



Feeding via a syringe Your baby can be fed tiny quantities of milk this way. Your expressed breast milk is nutritionally best for your baby, since your body makes it specifically for her, and it contains important antibodies that boost your baby's immature immunity.

Moving to a bottle If you want to bottle-feed, when your baby is strong enough to suck, you can use a smaller sized nipple that holds just a little milk at a time, which is easier for your tiny baby to manage.



Q When can I start expressing milk for my baby?

You can begin expressing milk as soon as you feel ready, the sooner the better. Even tiny amounts of breast milk from you contain important antibodies and nutrients specially formulated for your baby's needs. The sooner you start to express, the sooner you will establish a good milk supply, and the more likely you are to breast-feed successfully once your baby can coordinate sucking, breathing, and swallowing. While your baby is too small still to latch on to the breast, your milk can be fed to her via a tube.

Q When can I start to breast-feed my baby?

As your baby becomes stronger and gradually learns to coordinate sucking, breathing, and swallowing, you can start trying to bring her to the breast. A premature baby can take time to get the hang of breast-feeding, and even when she has developed her sucking technique, she may find it hard to latch on with her tiny mouth. At first, just give your baby short periods of time to practice latching on and sucking. She may not take much milk initially, and supplements may be needed. Be guided by the NICU nurses or doctors, who can help you ease your baby into breast-feeding. They will encourage you to hold her skin to skin so she is stimulated to breast-feed, and help her to build up her time on the breast slowly.

Q My baby spits up lot of milk. Why is this?

It's common for babies to bring up some milk after being fed, known as reflux, and it is especially common in premature babies. This is because the muscles that control the valve at the bottom of the esophagus, which stops food from coming back out of the stomach and up to the throat again, are less likely to be well developed. Keeping your baby upright during and after a feeding, spending time burping her, and, if you're bottle-feeding, switching to a slower-flow nipple to avoid her guzzling down milk too quickly, can all help to minimize the problem (see p.295 for more information and advice on dealing with reflux).